

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Mounjaro, Wegovy, Saxenda (liraglutide), Mysimba and Orlistat for the treatment of Obesity

Summary of NICE / NICE CKS Guidelines for Obesity (Weight Management in Adults)

Overview

- Obesity is defined as **excess body fat** that presents a risk to health.
- Diagnosis is based on **Body Mass Index (BMI)**, with **waist circumference** used to assess **central fat distribution** (especially in people with BMI <35).

Classification (Adults)

BMI (kg/m ²)	Classification
18.5–24.9	Healthy weight
25–29.9	Overweight
30–34.9	Obesity Class I
35–39.9	Obesity Class II
≥40	Obesity Class III (severe/morbid)

Adjust BMI thresholds for **South Asian, Black African, and some other minority ethnic groups** (e.g. action at BMI ≥23 for overweight, ≥27.5 for obesity).

Assessment

- Measure: **Weight, height, BMI, and waist circumference**
- Assess:
 - **Comorbidities:** type 2 diabetes, hypertension, sleep apnoea, CVD
 - **Lifestyle factors** (diet, activity, sleep, smoking, alcohol)

- **Readiness to change** and previous weight loss efforts
 - **Medication** contributing to weight gain
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Management Approach (Stepwise)

1. Lifestyle Intervention (First-Line for All)

- Multicomponent programme including:
 - **Calorie deficit** (aim for 600 kcal/day deficit or low-calorie diet ~1200–1500 kcal/day)
 - **Physical activity**: at least 150 minutes/week moderate-intensity
 - **Behavioural strategies**: goal-setting, self-monitoring, problem-solving
- Group-based programmes are often more effective

2. Pharmacological Treatment

- Consider **only if BMI ≥ 30** (or ≥ 27 with comorbidities) and **lifestyle measures have failed**.
- **First-line medication: Orlistat 120 mg TDS** with meals
 - Inhibits fat absorption
 - Continue only if $\geq 5\%$ weight loss after 3 months
- Monitor side effects (GI symptoms) and fat-soluble vitamin deficiency

3. Specialist Referral / Tier 3 Services

- Refer if:
 - **BMI ≥ 40** , or ≥ 35 with comorbidities
 - Failure of Tier 2 interventions
 - Consideration of pharmacological or surgical options

4. Bariatric Surgery (Tier 4)

- Consider for:
 - **BMI ≥ 40** , or ≥ 35 with comorbidities
 - After failure of non-surgical options
 - Must be committed to long-term follow-up
 - Includes gastric bypass, sleeve gastrectomy, gastric banding
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Follow-Up

- Regularly monitor **weight, BMI, waist circumference**, comorbidities
 - Support ongoing lifestyle changes
 - Relapse is common — plan for long-term support
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Prevention

- Encourage healthy weight in all adults
- Promote **healthy eating**, physical activity, and **weight awareness**
- Consider **brief interventions** opportunistically (e.g. GP appointments)

Patient Group Direction

for the supply/administration of

Mounjaro (tirzepatide)

for the treatment of

Obesity

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Mounjaro (tirzepatide) is indicated as an adjunct to a reduced-calorie diet and increased physical activity for weight management in adults with obesity (BMI ≥ 30 kg/m ²) or overweight (BMI ≥ 27 kg/m ²) with at least one weight-related comorbidity.
Inclusion criteria	- Adults aged 18 years and over. - BMI ≥ 30 kg/m², or BMI ≥ 27 kg/m² with weight-related comorbidities (e.g. hypertension, T2DM, dyslipidaemia). - Informed consent obtained. - No contraindications to tirzepatide.
Exclusion criteria	- Personal or family history of medullary thyroid carcinoma or MEN 2. - History of pancreatitis. - Severe gastrointestinal disease. - Pregnancy or breastfeeding. - Concurrent use of another GLP-1 receptor agonist.
Cautions	- Monitor for gastrointestinal side effects and advise on gradual dose escalation. - Counsel on lifestyle measures and regular monitoring of weight, glucose, and HbA1c. - Rotate injection sites to reduce irritation. - Reassess benefit-risk if no clinically meaningful weight loss after 6 months.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Mounjaro
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). May be stored below 30°C for up to 21 days. Do not freeze.
Route/method of administration	Subcutaneous injection in the abdomen, thigh, or upper arm.
Dose and frequency	Start with 2.5 mg once weekly. Increase gradually to a maintenance dose of 5–15 mg weekly as per clinical response and tolerability.
Quantity to be administered and/or supplied	One pre-filled pen per week, based on prescribed dose.
Maximum or minimum treatment period	Evaluate effectiveness after 12 weeks at maintenance dose; long-term use may be indicated.
Adverse effects	Nausea, vomiting, diarrhoea, decreased appetite, constipation, injection

	site reactions. Rare: pancreatitis, gallbladder disease. Refer to SmPC.
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

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responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		25/8/2025
Chris Pilkington	Pharmacist GPhC: 2046322		25/8/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Patient Group Direction

for the supply/administration of

Wegovy (semaglutide)

for the treatment of

Obesity

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Wegovy (semaglutide) is indicated as an adjunct to a reduced-calorie diet and increased physical activity for weight management in adults with obesity (BMI ≥ 30 kg/m ²) or overweight (BMI ≥ 27 kg/m ²) with at least one weight-related comorbidity.
Inclusion criteria	- Adults aged 18 years and over. - BMI ≥ 30 kg/m², or BMI ≥ 27 kg/m² with weight-related comorbidities (e.g. hypertension, T2DM, dyslipidaemia). - Informed consent obtained. - No contraindications to semaglutide.
Exclusion criteria	- Personal or family history of medullary thyroid carcinoma or MEN 2. - History of pancreatitis. - Severe gastrointestinal disease. - Pregnancy or breastfeeding. - Concurrent use of another GLP-1 receptor agonist.
Cautions	- Monitor for gastrointestinal side effects and counsel on gradual dose escalation. - Educate on lifestyle interventions and regular monitoring. - Rotate injection sites to reduce local irritation. - Reassess treatment if insufficient response after 6 months.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Wegovy (Semaglutide)
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). May be stored below 30°C for up to 6 weeks. Do not freeze.
Route/method of administration	Subcutaneous injection in the abdomen, thigh, or upper arm.
Dose and frequency	Start with 0.25 mg once weekly, titrating up every 4 weeks to a maximum of 2.4 mg weekly based on tolerance and response.
Quantity to be administered and/or supplied	One pre-filled pen per week, dose strength depending on treatment stage.
Maximum or minimum treatment period	Assess clinical benefit at 12 weeks on 2.4 mg dose; long-term treatment may be appropriate.
Adverse effects	Nausea, vomiting, diarrhoea, constipation, abdominal pain, injection site reactions. Rare: pancreatitis, gallbladder issues. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

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Name	Qualification	Signature	Date
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Chris Pilkington	Pharmacist GPhC: 2046322		25/8/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Patient Group Direction

for the supply/administration of

Saxenda (liraglutide)

for the treatment of

Obesity

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Saxenda (liraglutide) is indicated as an adjunct to a reduced-calorie diet and increased physical activity for weight management in adults with obesity (BMI ≥ 30 kg/m ²) or overweight (BMI ≥ 27 kg/m ²) with at least one weight-related comorbidity.
Inclusion criteria	- Adults aged 18 years and over. - BMI ≥ 30 kg/m², or BMI ≥ 27 kg/m² with weight-related comorbidities (e.g. hypertension, T2DM, dyslipidaemia). - Informed consent obtained. - No contraindications to liraglutide.
Exclusion criteria	- Personal or family history of medullary thyroid carcinoma or MEN 2. - History of pancreatitis. - Severe gastrointestinal disease. - Pregnancy or breastfeeding. - Concurrent use of another GLP-1 receptor agonist.
Cautions	- Titrate dose gradually to minimise GI side effects. - Counsel on adherence, lifestyle changes, and regular monitoring. - Rotate injection sites and monitor for local reactions. - Reassess need if <5% weight loss after 12 weeks at full dose.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Saxenda (Liraglutide)
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). May be stored below 30°C for up to 30 days. Do not freeze.
Route/method of administration	Subcutaneous injection in the abdomen, thigh, or upper arm.
Dose and frequency	Initiate at 0.6 mg once daily and increase by 0.6 mg increments weekly to a maximum of 3.0 mg once daily, based on tolerability and clinical response.
Quantity to be administered and/or supplied	One pre-filled pen per week, depending on dose and stage of titration.
Maximum or minimum treatment period	Evaluate response after 12 weeks at the full dose (3.0 mg). Discontinue if <5% weight loss.
Adverse effects	Nausea, diarrhoea, constipation, headache, vomiting. Rare: pancreatitis, gallbladder disorders. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

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Chris Pilkington	Pharmacist GPhC: 2046322		25/8/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Patient Group Direction

for the supply/administration of

Mysimba (naltrexone/bupropion)

for the treatment of

Obesity

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Mysimba (naltrexone/bupropion) is indicated as an adjunct to a reduced-calorie diet and increased physical activity for weight management in adults with a BMI ≥ 30 kg/m ² or BMI ≥ 27 kg/m ² with at least one weight-related comorbidity.
Inclusion criteria	- Adults aged 18 years and over. - BMI ≥ 30 kg/m², or BMI ≥ 27 kg/m² with comorbidities such as T2DM, hypertension, or dyslipidaemia. - Informed consent obtained. - No contraindications to naltrexone or bupropion.
Exclusion criteria	- History of seizure disorders or current seizure risk. - Uncontrolled hypertension. - Eating disorders (e.g. bulimia or anorexia nervosa). - Pregnancy or breastfeeding. - Current use of opioids or opioid withdrawal. - Severe hepatic or renal impairment.
Cautions	- Titrate dose to minimise nausea and side effects. - Monitor BP and mental health symptoms during treatment. - Advise on adherence, diet, and exercise. - Discontinue if no $\geq 5\%$ weight loss after 16 weeks at full dose.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Mysimba (naltrexone/bupropion)
Legal category	POM
Storage	Store below 25°C. Store in original packaging to protect from moisture.
Route/method of administration	Oral administration. Swallow tablets whole, do not cut, chew, or crush.
Dose and frequency	Start with 1 tablet daily, increasing weekly by 1 tablet/day up to a maintenance dose of 2 tablets twice daily (total of 4 tablets per day) from week 4 onward.
Quantity to be administered and/or supplied	Initial supply for titration phase; subsequent based on maintenance dosing (4 tablets/day).
Maximum or minimum treatment period	Evaluate weight loss at 16 weeks. Discontinue if $<5\%$ weight loss at full dose.
Adverse effects	Nausea, constipation, headache, dizziness, dry mouth, insomnia. Rare: increased blood pressure, seizures. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

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Name of professional group signatory	Job title & name of organisation	Signature	Date
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Chris Pilkington	Pharmacist GPhC: 2046322		25/8/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Patient Group Direction

for the supply/administration of

Orlistat

for the treatment of

Obesity

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Orlistat is indicated as an adjunct to a mildly hypocaloric diet for weight loss in adults with a BMI ≥ 30 kg/m ² , or ≥ 28 kg/m ² with associated risk factors such as type 2 diabetes, hypertension, or dyslipidaemia.
Inclusion criteria	- Adults aged 18 years and over. - BMI ≥ 30 kg/m², or BMI ≥ 28 kg/m² with weight-related risk factors. - Informed consent obtained. - No contraindications to orlistat.
Exclusion criteria	- Chronic malabsorption syndrome or cholestasis. - Pregnancy or breastfeeding. - History of hyperoxaluria or oxalate nephrolithiasis. - Concurrent use of ciclosporin, warfarin (without monitoring), or acarbose. - BMI < 28 kg/m².
Cautions	- Advise patients to follow a nutritionally balanced, reduced-calorie diet with ~30% calories from fat. - Recommend multivitamin supplementation (A, D, E, K) if used long term. - Counsel on managing gastrointestinal side effects. - Reassess treatment if $< 5\%$ weight loss after 12 weeks of therapy.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Orlistat
Legal category	POM
Storage	Store below 25°C. Store in the original package to protect from moisture.
Route/method of administration	Oral administration with water, immediately before, during, or up to one hour after each main meal.
Dose and frequency	120 mg three times daily with each main meal containing fat.
Quantity to be administered and/or supplied	90 capsules for 30 days, adjustable based on treatment plan.
Maximum or minimum treatment period	Evaluate after 12 weeks. Discontinue if weight loss $< 5\%$ from baseline.
Adverse effects	Oily stools, flatulence with discharge, faecal urgency, abdominal pain, headache. Rare: liver dysfunction. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and
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	inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates

Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		25/8/2025
Chris Pilkington	Pharmacist GPhC: 2046322		25/8/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025